

Clinical Guideline Handbook (Generated from clinicalguide-ai dataset)

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Purpose: Condition-by-condition pathway guidance formatted for n8n RAG reasoning.

Safety note: This document is generated from source dataset rows and may include shorthand/typos from source. Use as decision support, not as a replacement for clinician judgment or local protocols.

Total source rows: 3766 | Conditions found: 11

- hotness of body (1546 rows)
- Elevated BPs in Pregnancy (1011 rows)
- EPIGASTRIC PAIN (476 rows)
- PV BLEEDING (265 rows)
- BURNS (111 rows)
- CHEST TRAUMA (84 rows)
- HEAD INJURY (80 rows)
- ABDOMINAL INJURY (62 rows)
- Sheet2 (54 rows)
- WHEEZE (52 rows)
- HYPONATREMIA (25 rows)

1. CONDITION: hotness of body

Assessment and Intake Parameters:

- Presenting complaints
- Duration
- Pre- Treatment
- Age
- Signs & symptoms
- Investigations
- Results
- Additional tests(sample collected in out patient)
- Diagnosis
- Plan
- SUPPORTIVE MANAGEMENT
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed
- Hotness of Body
- <7 days
- no
- >60 days old
- No other sign
- FHG + malaria AG
- Normal FHG+ Malaria negative
- Acute febrile illness R/O viral, atypical infections
- see as OUT PT ,TCA 7 days
- Expose patient , tepid sponge
- amoxycillin+ clavulanate over 12years 375-750mg, below 12 years 25-50mg/kg/day, paracetamol 10mg/kg/dose
- fleming, paralief for children, panadol advance

- Granulocytosis + malaria negative

- acute bacterial infection
- lymphocytosis+ malaria negative
- acute viral infection
- hb< 10g/dl + malaria negative
- PBF+ Reticulocyte count+ HIV+GXM
- anaemia + acute febrile illness
- admit patient for further work up and possible transfusion
- Expose patient , tepid sponge , secure an IV line neck support, reassure the relatives
- paracetamol 10mg/kg/dose, ceftriaxone 50mg/kg/dose, transfuse if symptomatic, ranferon
- Rocephin,paraconica 1g
- Zoxon 2gm, Paraconica 1gm
- Normal FHG+ Malaria positive
- Malaria
- see as an outpatient and TCA 3 days
- amoxycillin+ clavulanate over 12years 375-750mg, below 12 years 25-50mg/kg/day, paracetamol 10mg/kg/dose, artemether/lumefantrine per kg body weight
- fleming, paralief for children, panadol advance, ACTM 20/120
- ACTM 20/120, FLEMING, paralief, panadol advance
- Granulocytosis + malaria positive
- malaria+ bacterial infection
- admit patient for parenteral treatment
- Artesunate 2.4mg/kg loading dose +paracetamol 10 mg/kg/dose, ceftriaxone 50mg/kg/dose
- Rocephin,paraconica 1g, Artesun
- lymphocytosis+ malaria positive
- malaria + viral infection
- lymphopenia+ malaria positive
- LFTs + PBF
- severe malaria
- Zoxon 2gm, Paraconica 1gm, artesun
- hb< 10g/dl + malaria positive
- LFTs + PBF+ GXM+ reticulocyte count
- severe malaria + anaemia
- admit patient for parenteral treatment and further work up
- Artesunate2.4mg/kg/dose, paracetamol, 10mg/kg/dose, ceftriaxone 100mg/kg/dose transfuse if < 5g/dl and give ranferon at HB> 6g/dl
- Hallucinations/delusions/abnormal posturing
- FHG + Malaria Ag+ U/E/C + RBS
- normal FHG, normal UEC, mal neg, normal RBS
- CT head, HIV,consider lp in ward
- probable dx-viral encephalitis, meningitis, drug withdrawal,psychosomatic disorder
- Admit sedate(with midazolam 0.15mg/kg) if there is a bizarre behaviours and consult
- Iv Ceftriaxone 100mg/kg BD Max dose 2g, paracetamol 10mg/kg/dose
- Granulocytosis , mal neg, normal UEC, normal RBS
- Blood culture, consider lp in ward
- probable dx- meningitis, Brain abscess,
- lymphocytosis , mal neg, normal UEC, normal rbs
- CT head, consider lp in ward
- probable dx- meningitis, encephalitis, brain lymphoma, neuroma

RAG-Ready Clinical Pathway Narrative:

For hotness of body, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

2. CONDITION: Elevated BPs in Pregnancy

Assessment and Intake Parameters:

- Presentation
- Pregnant
- Signs & symptoms
- Examination Findings
- Investigations
- Results
- Additional tests(sample collected in out patient)
- Diagnosis
- Plan
- SUPPORTIVE MANAGEMENT
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed
- ELEVATED BP $\geq$ 140/90mmHg
- >20 weeks
- WITH ANY OF: signs of target organ damage (Severe headache with blurring of vision, Epigastric pain, Oliguria , signs , liver tenderness
- FHG + Urinalysis +U/E/C+LFTs + Coagulation profile
- Normal FHG + Normal Transaminases + Normal U/E/C + Normal coagulation profile + No Proteinuria
- Obstetric U/S + Uric acid + PBF
- Severe Pre - Eclampsia
- Admit and call gynaecologist immediately
- reassure patient, place the patient in left lateral position, restrict fluids to 80mls per hr of ringers lactate avoid DNS or Dextrose.
- if BP $> 160/110$ mm/hg give IV hydralazine 5mg slowly over 10 minutes , Give loading 4 GM of 20% MGSO4 over 15 minutes then continue with 1 GM hourly for 24 hours as you monitor the respiratory rate, urine out put and patellar reflexes. And continue for 24 hours after delivery whichever comes first.
- Apresolin 20mg inj, Nalapsin 4gm injection
- Hb <10 g/dl + Normal Transaminases + Normal U/E/C + Normal coagulation profile+ No Proteinuria
- Thrombocytopenia < 100 +Normal Transaminases + Normal U/E/C + Normal coagulation profile+ No Proteinuria
- Normal FHG + Elevated Transaminases + Normal U/E/C + Normal coagulation profile+ No Proteinuria
- Hb <10 g/dl + Elevated Transaminases + Normal U/E/C + Normal coagulation profile+ No Proteinuria
- Thrombocytopenia < 100 + Elevated Transaminases + Normal U/E/C + Normal coagulation profile+ No Proteinuria
- Normal FHG + Normal Transaminases + Deranged U/E/C + Normal coagulation profile+ No Proteinuria
- Hb <10 g/dl + Normal Transaminases + Deranged U/E/C + Normal coagulation profile+ No Proteinuria
- Thrombocytopenia < 100 +Normal Transaminases + Deranged U/E/C + Normal coagulation profile+ No Proteinuria
- Normal FHG + Normal Transaminases + Normal U/E/C +Abnormal coagulation profile+ No Proteinuria
- Hb <10 g/dl + Normal Transaminases + Normal U/E/C +Abnormal coagulation profile+ No Proteinuria
- Thrombocytopenia < 100 +Normal Transaminases + Normal U/E/C + Abnormal coagulation profile+ No Proteinuria
- Normal FHG + Elevated Transaminases + Deranged U/E/C + Normal coagulation profile+ No Proteinuria
- Hb <10 g/dl + Elevated Transaminases +Deranged U/E/C + Normal coagulation profile+ No Proteinuria
- Thrombocytopenia < 100 + Elevated Transaminases + Deranged U/E/C + Normal coagulation profile+ No Proteinuria
- Normal FHG + Elevated Transaminases + Deranged U/E/C + Abnormal coagulation profile + Proteinuria
- Hb <10 g/dl + Elevated Transaminases +Deranged U/E/C +Abnormal coagulation profile+ Proteinuria
- Thrombocytopenia < 100 + Elevated Transaminases + Deranged U/E/C + Abnormal coagulation profile+ Proteinuria
- Normal FHG + Normal Transaminases + Normal U/E/C + Normal coagulation profile + Proteinuria
- Hb <10 g/dl + Normal Transaminases + Normal U/E/C + Normal coagulation profile+ Proteinuria
- Thrombocytopenia < 100 +Normal Transaminases + Normal U/E/C + Normal coagulation profile+ Proteinuria

- Normal FHG + Elevated Transaminases + Normal U/E/C + Normal coagulation profile+ Proteinuria
- Hb <10 g/dl + Elevated Transaminases + Normal U/E/C + Normal coagulation profile+ Proteinuria
- Thrombocytopenia < 100 + Elevated Transaminases + Normal U/E/C + Normal coagulation profile+ Proteinuria
- Normal FHG + Normal Transaminases + Deranged U/E/C + Normal coagulation profile+ Proteinuria
- Hb <10 g/dl + Normal Transaminases + Deranged U/E/C + Normal coagulation profile+ Proteinuria
- Thrombocytopenia < 100 +Normal Transaminases + Deranged U/E/C + Normal coagulation profile+ Proteinuria
- Normal FHG + Normal Transaminases + Normal U/E/C +Abnormal coagulation profile+ Proteinuria
- Hb <10 g/dl + Normal Transaminases + Normal U/E/C +Abnormal coagulation profile+ Proteinuria
- Thrombocytopenia < 100 +Normal Transaminases + Normal U/E/C + Abnormal coagulation profile+ Proteinuria
- Normal FHG + Elevated Transaminases + Deranged U/E/C + Normal coagulation profile+ Proteinuria
- Hb <10 g/dl + Elevated Transaminases +Deranged U/E/C + Normal coagulation profile+ Proteinuria
- Thrombocytopenia < 100 + Elevated Transaminases + Deranged U/E/C + Normal coagulation profile+ Proteinuria
- WITHOUT ANY OF:signs of target organ damage (Severe headache with blurring of vision, Epigastric pain, Oliguria , signs , liver tenderness
- Normal FHG + Normal Transaminases + Normal U/E/C + No Proteinuria
- Obstetric scan
- Gestational hypertension/Mild Pre Eclampsia
- reassure patient, place the patient in left lateral position.
- Methyl dopa 250 mg TID
- Aldomet 250mg tablets
- Hb <10 g/dl + Normal Transaminases + Normal U/E/C + No Proteinuria
- Obstetric scan, PBF
- Gestational hypertension + Anaemia/ Mild Pre Eclampsia
- Normal FHG + Elevated Transaminases + Normal U/E/C + No Proteinuria
- Abdominal scan + Obstetric scan
- Severe Pre eclampsia
- Hb <10 g/dl + Elevated Transaminases + Normal U/E/C + No Proteinuria
- Severe Pre eclampsia , Anaemia
- Normal FHG + Normal Transaminases + Deranged U/E/C +No Proteinuria
- Hb <10 g/dl + Normal Transaminases + Deranged U/E/C + No Proteinuria

RAG-Ready Clinical Pathway Narrative:

For Elevated BPs in Pregnancy, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

3. CONDITION: EPIGASTRIC PAIN

Assessment and Intake Parameters:

- Presenting complaints
- Signs & symptoms
- Supportive Management
- Investigations
- Results
- ADDITIONAL TESTS
- Diagnosis
- Plan
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult

- Total Cost Paed

- EPIGASTRIC PAIN

- bloody Vomitus WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg
- See appendix (x) for hypovolemic shock resuscitation
- FHG + U/E/C + LFTs + H- Pylori Ag
- Normal FHG + Normal Transaminases + Normal U/E/C + H-Pylori -VE
- Endoscopy, GXM
- probable dx- Upper GI bleeding. With shock
- Admit and urgent surgical consult
- start on fluid resuscitation (see appendix x), iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Trenexamic 10mg/kg/dose/TID
- Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj, Hemsamic 500mg injection
- Klacid 500mg injection,Flemin 1.2gm injection, Zantac 50mg inj, Hemsamic 500mg injection
- Hb <10 g/dl + Normal Transaminases + Normal U/E/C+ H-Pylori -VE
- probable dx- Upper GI bleeding, Anaemia With shock
- Thrombocytopenia < 100+ Normal Transaminases + Normal U/E/C + H-Pylori -VE
- Endoscopy ,coagulation profile, GXM
- Probable dx- Upper GI bleeding with shock R/O a coagulopathy
- Normal FHG + Elevated Transaminases + Normal U/E/C + H-Pylori -VE
- Endoscopy ,coagulation profile, Abdominal ultrasound, GXM
- probable dx- Liver failure, oesophageal varices.
- Hb <10 g/dl + Elevated Transaminases + Normal U/E/C + H-Pylori -VE
- Endoscopy ,coagulation profile, Abdominal ultrasound,GXM
- Thrombocytopenia < 100+ Elevated Transaminases + Normal U/E/C + H-Pylori -VE
- probable dx- Liver failure, oesophageal varices. Coagulopathy.
- Normal FHG + Normal Transaminases + Deranged U/E/C + H-Pylori -VE
- Endoscopy , Abdominal ultrasound,GXM, urinalysis
- probable dx- Shock, R/O AKI
- Hb <10 g/dl + Normal Transaminases + Deranged U/E/C + H-Pylori -VE
- Endoscopy , Abdominal ultrasound, coagulation profile.,GXM, urinalysis
- probable dx- Shock, R/O AKI, Anaemia.
- Thrombocytopenia < 100+ Normal Transaminases + Deranged U/E/C + H-Pylori -VE
- Endoscopy , Abdominal ultrasound, coagulation profile,GXM, urinalysis
- probable dx-shock,R/O AKI,R/O coagulopathy
- Normal FHG + Normal Transaminases + Normal U/E/C + H-Pylori +VE
- Endoscopy , Abdominal ultrasound,GXM
- Probable dx- gastritis, duodenitis, Upper GI bleeding in shock
- Hb <10 g/dl + Normal Transaminases + Normal U/E/C+ H-Pylori +VE
- Probable dx- gastritis, duodenitis, Upper GI bleeding in shock,Anaemia
- Thrombocytopenia < 100+ Normal Transaminases + Normal U/E/C + H-Pylori +VE
- Endoscopy , Abdominal ultrasound,GXM coagulation
- Probable dx- gastritis, duodenitis, Upper GI bleeding in shock, coagulopathy.
- Normal FHG + Elevated Transaminases + Deranged U/E/C + H-Pylori +VE
- Endoscopy , Abdominal ultrasound,GXM coagulation , urinalysis
- probable diagnosis: upper GIT bleeding in shock R/O liver, kidney failure, gastritis, duodenitis
- Hb <10 g/dl + Elevated Transaminases + Deranged U/E/C+ H-Pylori +VE
- Endoscopy , Abdominal ultrasound,GXM coagulation ,urinalysis
- Thrombocytopenia < 100+ Elevated Transaminases + Deranged U/E/C + H-Pylori +VE
- probable diagnosis: upper GIT bleeding in shock R/O liver, kidney failure, gastritis, duodenitis, coagulopathy
- bloody Vomitus WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg
- start IV normal saline as you consult
- probable dx: upper GIT bleeding find cause

- Start on, iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Tranexamic acid 10mg/kg/dose TID
- probable dx: upper GIT bleeding R/O liver failure, esophageal varises
- probable dx: upper GIT bleeding R/O liver failure, esophageal varises, coagulopathy
- probable dx: upper GIT bleeding R/O AKI
- probable dx: upper GIT bleeding R/O AKI, Coagulopathy
- probable dx: upper GIT bleeding R/O gastritis, duodenitis
- probable dx: upper GIT bleeding R/O gastritis, duodenitis, coagulopathy
- probable dx: upper GIT bleeding R/O AKI, liver failure
- Non Bloody VOMITUS, WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg
- Endoscopy ,Abdominal ultrasound
- Probable Dx- Gastritis
- start on fluid resuscitation (see appendix x), iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,

RAG-Ready Clinical Pathway Narrative:

For EPIGASTRIC PAIN, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

4. CONDITION: PV BLEEDING

Assessment and Intake Parameters:

- Presenting complaints
- Duration
- Pre- Treatment
- Pregnant
- Signs & symptoms
- Examination Findings
- Investigations
- Results
- Additional tests(sample collected in out patient)
- Diagnosis
- Plan
- SUPPORTIVE MANAGEMENT
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed
- Per Vaginal Bleeding
- NO
- with sign of shock(Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg
- NO Local bleeding lesions
- FHG + Pelvic U/S as you continue fluid resuscitation
- Normal FHG

- Hormonal profile, U/E/C,
- Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause
- Admit + Gynecological Review
- See appendix (x) for hypovolemic shock resuscitation
- Tranexamic acid 1g IV tds, IV Ceftriaxone 50mg/kg, IV Metronidazole 7.5mg/kg
- Hemsamic 50mg inj, Rocephin 1gm, Flagyl infusion
- Hemsamic 50mg inj, Zoxon 1gm, Flagyl infusion
- Granulocytosis
- Hormonal profile, U/E/C, + LFTs
- Thrombocytopenia
- Hormonal profile, U/E/C, Coagulation profile + PBF
- Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause , ITP, VWD,
- Hb < 10 g/dl
- Hormonal profile, U/E/C, PBF
- Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause, Anemia
- <20 weeks
- FHG + OBS U/S as you continue fluid resuscitation
- U/E/C
- Probable dx: Abortion with SHOCK
- U/E/C + LFTs
- Probable dx: Abortion with SHOCK, DIC
- U/E/C + PBF + Coagulation Profile
- U/E/C + PBF +
- >20 weeks
- Probable dx: APH with SHOCK
- Probable dx: APH with SHOCK, DIC
- WITHOUT signs of shock(Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg & Hemodynamically stable
- FHG + Pelvic U/S
- Probable dx: with Abnormal Uterine Bleeding ? Cause
- Admit for Gynecological Review IF ACTIVELY BLEEDING If not bleeding see as OP & TCA in GOPC
- Reassure patient, Start normal saline if actively bleeding as you consult
- OUT PATIENT: Tranexamic acid 1g PO tds, OP Cefuroxime 50mg/kg, PO Metronidazole 7.5mg/kg, IN PATIENT Tranexamic acid 1g IV tds IV Ceftriaxone 50mg/kg, IV Metronidazole 7.5mg/kg
- Hemsamic 500mg caps, Axacef 500mg tab, Flagyl 400mg tablets
- Probable dx: Abortion
- Probable dx: APH

RAG-Ready Clinical Pathway Narrative:

For PV BLEEDING, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

5. CONDITION: BURNS

Assessment and Intake Parameters:

- Presentation
- ADMISSION CRITERIA IRRESPECTIVE OF SIGNS & SYMPTOMS
- Age
- TBSA
- Thickness
- Supportive Management
- Investigations
- Diagnosis
- Plan

- Treatment

- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed

- PATIENT WITH BURNS

- Partial thickness burns greater than 10% total body surface area (TBSA). 2. Burns that involve the face, hands, feet, genitalia, perineum, or major joints. 3. Third degree burns in any age group. 4. Electrical burns, including lightning injury. 5. Chemical burns. 6. Inhalation injury. 7. Burn injury in patients with preexisting medical disorders that could complicate management, prolong recovery, or affect mortality. 8. Any patient with burns and concomitant trauma (such as fractures) in which the burn injury poses the greatest risk of morbidity or mortality. . 9. Extremes of age <5 & >60 10. Burn injury in patients who will require special social, emotional, or rehabilitative intervention 12. Circumfrential burns

- 10 years - 50 years

- <15%

- Partial

- IN SEVERE BURNS DO ACBE remove all burned clothing, immerse the site in cold water for 30 minutes, douse with cool water, apply clean wraps on burned area, administer tetanus prophylaxis, gently cleanse the burn with 0.25% chlorhexidine solution, DO NOT use alcohol-based solutions, Gentle scrubbing will remove the loose necrotic tissue. Apply a thin layer of antibiotic cream (silver sulfadiazine). Dress the burn with petroleum gauze and dry gauze

- FHG + U/E/C

- MILD BURN INJURY

- Treat as OP & TCA 3 days

- Flucloxacillin 50mg/kg/dose + Paracetamol 10mg/kg/dose + silver sulphur diazine cream

- Adult Floxapen 500mg caps, Panadol advance, silverex 500gm, 25gm, Paed floxapen 125mg syrup, panadol baby and infant, silverex 500gm, 25gm

- Flucloxacillin 500mg caps, cetamol 500mg tablets, silverex 500gm, 25gm, Paed floxapen 125mg syrup, paralief syrup and infant, silverex 500gm, 25gm

- < 10 Years OR > 50 Years

- <10%

- Irrespective

- <2%

- Full

- Admit for Surgical review

- IV Flucloxacillin 50mg/kg/dose + IV Metronidazole 7.5mg/kg + Pethidine 50mg IM + Tetracycline eye ointment in facial burns, Pethidine 50mg IM, Omeprazole 4mg/kg/dose, silver sulphur diazine cream

- Floxapen 500mg inj, Flagyl infusion, pethidine 50mg, tetracycline eye ointment, losec 40mg injection, silverex cream 500gm

- Floxapen 500mg inj, Flagyl infusion, pethidine 50mg, tetracycline eye ointment, Risek 40mg injection, silverex cream 500gm

- 10 Years - 50 Years

- 15-25%

- IN SEVERE BURNS DO ACBE remove all burned clothing, immerse the site in cold water for 30 minutes, douse with cool water, apply clean wraps on burned area, administer tetanus prophylaxis, gently cleanse the burn with 0.25% chlorhexidine solution, DO NOT use alcohol-based solutions, Gentle scrubbing will remove the loose necrotic tissue. Apply a thin layer of antibiotic cream (silver sulfadiazine). Dress the burn with petroleum gauze and dry gauze SEE APPENDIX 12 FOR FLUID MANAGEMENT IN BURNS

- MODERATE BURN INJURY

- IV Flucloxacillin 50mg/kg/dose + IV Metronidazole 7.5mg/kg + Pethidine 50mg IM + Tetracycline eye ointment in facial burns, Pethidine 50mg IM, Omeprazole 4mg/kg/dose, silver sulphur diazine cream

- 10 - 20%

- 2-10%

- >25%
- FHG + U/E/C + LFTs + Coagulation profile
- SEVERE BURN INJURY
- >20%
- >10%

RAG-Ready Clinical Pathway Narrative:

For BURNS, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

6. CONDITION: CHEST TRAUMA

Assessment and Intake Parameters:

- Presenting complaints
- Signs & symptoms
- PERCUSSION NOTE
- Supportive Management
- Investigations
- Additional tests(sample collected in out patient)
- Diagnosis
- Plan
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed
- Trauma to the Chest + Difficulty in breathing.
- WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg
- Normal
- See appendix (x) for hypovolemic shock resuscitation,give ovygen, Do Assess and manage Airway, Circulation, Breathing, Disability. As you consult .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK
- chest x-ray
- FHG, U/E/C. Abdominal scan especially if # is in 11th and 12th ribs or with associated abdominal tenderness
- Dx- Blunt chest trauma in shock.
- Admit and consult immidiately
- IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose
- Rocephin 1gm injection, Flagy infusion, Paraconica 1gm injection
- Zoxon 1gm injection, Flagy infusion, Paraconica 1gm injection
- WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg
- Reassure patient, maintenance IVF,.IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK
- Dx- Blunt chest trauma
- Hyperresonant
- See appendix (x) for hypovolemic shock resuscitation,give ovygen, Do Assess and manage Airway, Circulation, Breathing, Disability,isert a large bore carnular at 2nd intercostal space mid clavicular line,.IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK

- Dx- Blunt chest trauma with pneumothorax

- Reassure patient, maintenance IVF, give oxygen, Do Assess and manage Airway, Circulation, Breathing, Disability, insert a large bore cannula at 2nd intercostal space mid clavicular line urgent surgical review, .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK

- Dull

- See appendix (x) for hypovolemic shock resuscitation, give oxygen, Do Assess and manage Airway, Circulation, Breathing, Disability as you consult for chest tube insertion, .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK

- Dx- Blunt chest trauma with hemothorax

- Reassure patient, maintenance IVF, .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK

RAG-Ready Clinical Pathway Narrative:

For CHEST TRAUMA, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

7. CONDITION: HEAD INJURY

Assessment and Intake Parameters:

- Presenting complaints
- GCS
- Signs & symptoms
- Supportive Management
- Investigations
- Additional tests(sample collected in out patient)
- Diagnosis
- Plan
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed
- Trauma to the head
- GCS > 13
- WITH:-Convulsions, Confusion, Otorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs , extremities of age < 5 years > 60 years
- Assess Airway , Circulation, Breathing, Disability and Exposure(ACBDE) Arrest Bleeding(If Present) , if in shock see appendix (x)
- Order an urgent CT scan Head,
- U/E/C, RBS, LFTS FHG, Alcohol toxicity levels.
- Dx- Mild Head injury
- Admit and immediate surgical consult
- IV ceftriaxone 100mg/kg/dose , Paracetamol 10mg/kg/dose, Phenytoin 20mg/kg to run over 15 mins with ECG & BP monitoring, TT 0.5mg IM SAT in > 5 years old
- Rocephin 2gm, Paraconica 1gm, Epanutin 250mg inj, Tetanus injection
- Zoxon 2gm, Paraconica 1gm, Epanutin 250mg inj, Tetanus injection
- GCS 8-12
- Regardless of:-Convulsions, Confusion, Otorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs extremities of age < 5 years > 60 years
- U/E/C, RBS, LFTS FHG, Alcohol toxicity levels

- Dx- Moderate Head injury

- GCS < 8
- Regardless of: -Convulsions, Confusion, Otorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs extremities of age < 5 years > 60 years
- Dx- severe Head injury
- WITHOUT:-Convulsions, Confusion, Otorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs , extremities of age < 5 years > 60 years
- Skull X-Ray
- Dx: Mild Head Injury
- Admit for Observation
- Dx: Moderate Head Injury
- Dx: Severe Head Injury

RAG-Ready Clinical Pathway Narrative:

For HEAD INJURY, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

8. CONDITION: ABDOMINAL INJURY

Assessment and Intake Parameters:

- Presenting complaints
- Penetrating
- Signs & symptoms
- Supportive Management
- Investigations
- Additional tests(sample collected in out patient)
- Diagnosis
- Plan
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed
- Abdominal Injury
- NO
- WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg
- See appendix (x) for hypovolemic shock resuscitation
- Focused Assessment Sonography in Trauma (FAST U/S) as you consult
- FHG + U/E/C + GXM + LFTs
- Blunt Abdominal injury with SHOCK
- Admit and immediate surgical consult
- IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose
- Rocephin 1gm injection, Flagyl infusion, paraconica 1gm
- Zoxon 1gm injection, Flagyl infusion, paraconica 1gm
- YES
- See appendix (x) for hypovolemic shock resuscitation, DO NOT REMOVE any impinged penetrating object, if no impinged object PACK WITH STERILE GAUZE
- Penetrating Abdominal injury with SHOCK
- IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose , TT 0.5mg IM SAT in >

5 years old

- Rocephin 1gm injection, Flagyl infusion, paraconica 1gm tetanus injection
- Zoxon 1gm injection, Flagyl infusion, paraconica 1gm tetanus injection
- WITHOUT signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg
- Reassure patient, maintenance IVF,
- FHG + U/E/C + LFTs + GXM
- Blunt Abdominal injury
- Admit for observation
- Reassure patient, maintenance IVF,DO NOT REMOVE any impinged penetrating object, if no impinged object PACK WITH STERILE GAUZE
- Penetrating Abdominal injury

RAG-Ready Clinical Pathway Narrative:

For ABDOMINAL INJURY, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

9. CONDITION: Sheet2

Assessment and Intake Parameters:

- RISK FACTORS FOR PPH
- PRENATAL
- ANTEPARTUM
- DURING ADMISSION
- INTRAPARTUM
- MEDIUM
- HIGH RISK
- Suspected previa/increta/percreta
- Placent previa
- prior C/S (MEDIUM)
- placenta previa
- Chorioamnionitis
- NEW Active bleeding
- BMI> 50
- placenta accreta
- Uterine surgery
- suspected Accreta/Percreta
- prolonged oxytocin > 24 hours
- 2/> medium risk factors(admission OR intrapartum)
- prior classical C/S
- multiple Gestation
- Platelet < 70,000
- prolonged 2nd stage
- prior myomectomy
- > 4 prior births
- Active bleeding
- large myomas
- known coagulopathy
- EFW> 4000g
- 2/> medium risk
- Obesity BMI> 40
- Himatocrit < 30%
- Presenting complaints
- Signs & symptoms

- Investigations

- Diagnosis
- Plan
- SUPPORTIVE MANAGEMENT
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed
- PV bleeding > 500mls vaginal / > 1000mls C/S WITH Normal VITAL SIGNS and Lab values
- FHG +U/E/C + GXM
- PPH stage 1
- Continual bleeding expected Blood loss upto 1500 mls OR 2 Uterotonics with normal lab values and vital signs

RAG-Ready Clinical Pathway Narrative:

For Sheet2, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

10. CONDITION: WHEEZE

Assessment and Intake Parameters:

- Presenting complaints
- Signs & symptoms
- Investigations
- Diagnosis
- Plan
- SUPPORTIVE MANAGEMENT
- Treatment
- Brand Name
- CASH IP(BRAND)
- Consultation
- Laboratory
- Imaging
- Pharmacy Adult
- Pharmacy Paeds
- Total Cost Adult
- Total Cost Paed
- Wheezing with Difficulty in Breathing
- WITH ANY OF: While walking, Talks in Alertness, Can lie down, SentencesMay be agitated, Increased Respiratory rate,no use of accessory muscles,wheeze often only end expiratory, Pulse Rate <100, Pulsus paradoxus, Absent/ <10 mm Hg
- FHG
- MILD ASTHMATIC ATTACK
- TREAT AS OP & TCA 24HRS,
- Reassure patient/ guardian, administer oxygen to maintain SPO2 > 90%, Give health education See appendix (XII)
- Nebulize with Salbutamol 1Mo - 11 yrs 2.5mg, > 11 years 5mg can be repeated up to 3 doses in the 1st hour + Prednisolone 1mg/kg daily for 1/52
- Ventolin nebulizing solution, prednisolone 5mg tablets

- **WITH ANY OF:** While at rest (infant—softer, shorter cry, difficulty feeding), Commonly uses accessory muscles, wheeze Loud throughout exhalation, Pulse Rate 100-120 Age 2 -12Mo <160/min, 1-2 Years <120/min 2-8 <110/min, Pulsus paradoxus May be present 10–25 mm Hg
- FHG + U/E/C
- MODERATE ASTHMATIC ATTACK
- ADMIT & CALL PHYSICIAN IMMEDIATELY
- Nebulize with Salbutamol 1Mo - 11 yrs 2.5mg, > 11 years 5mg can be repeated up to 3 doses in the 1st hour + IV Ipratropium 200mg + IV Hydrocortisone 50mg(1-5 yrs), 100mg (6-12 yrs),
- Ventolin nebulizing solution, Atrovent 250mg nebules, Hydrocortison 100mg inj
- **WITH ANY OF:** While at rest (infant—stops feeding), Sits upright, Words Usually agitated, wheeze loud throughout inhalation and exhalation, Pulse Rate >120, Pulsus paradoxus, Often present . 25 mm Hg (adult), 20–40 mm Hg (child)
- FHG + U/E/C + BGAs
- SEVERE ASTHMATIC ATTACK
- **WITH:** Drowsy / Confused, paradoxical thoracoabdominal movement, absence of wheeze, Bradycardia, absence of pulsus paradoxus
- ACTUAL/ IMPENDING RESPIRATORY FAILURE
- ADMIT IN CRITICAL CARE & CALL PHYSICIAN IMMEDIATELY
- INTUBATE PATIENT Reassure patient/ guardian, administer oxygen to maintain SPO2 > 90%, Give health education See appendix (XII)

RAG-Ready Clinical Pathway Narrative:

For WHEEZE, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

11. CONDITION: HYPONATREMIA

Assessment and Intake Parameters:

- HYPONATRAEMIA TREATMENT.
- (Serum Na < 130 mmol/L)
- signs and symptoms
- nausea, malaise, lethargy, decreased level of consciousness, headache, if severe (seizures and coma), overt neurologic symptoms (<115 mEq/L)
- CLASSIFICATION
- 1. Hypovolemic- decrease in total body water with greater decrease in total body sodium
- 2. Euvolemic- normal body sodium with increase in total body water
- 3. Hypervolemic- increase in total body sodium with increase in total body water
- LAB WORKUPS
- MANAGEMENT
- In the event of a seizure, coma or suspected cerebral herniation as a result of hyponatremia, IV 3% hypertonic saline should be administered as soon as possible according to the following guide:
 - a. Administer 3% hypertonic saline 100-150cc IV over 5-10min
 - b. Reassess patient
 - c. If the patient does not improve clinically after the first bolus, repeat a second bolus of hypertonic saline.
 - d. STOP ALL FLUIDS after the second bolus of hypertonic saline to avoid raising the serum sodium any further
- What if hypertonic saline is not readily available? Administer one amp of Sodium Bicarbonate over 5min.
- Defend the Intravascular Volume
- In order to defend the intravascular volume a determination an assessment of the patient's volume status must occur. Is the patient hypovolemic, euvolemic or hypervolemic?
- Although volume status is difficult to assess with any accuracy at the bedside, a clinical assessment with attention to the patient's history, heart rate, blood pressure, JVP, the presence of pedal and sacral edema.
- Make a rough determination of whether the patient is significantly hypovolemic (requiring fluid resuscitation) or significantly hypervolemic (requiring fluid restriction +/- diuretics).

- In a patient who is hypovolemic and hyponatremic, the priority is to restore adequate circulating volume
- For hypovolemic/hyponatremic patient, be mindful of the Na concentration of the fluid that you have chosen. Ringer's lactate has a sodium concentration of 128mmol/L which will be more ISOTONIC to the hyponatremic patient.
- Administering Ringer's lactate will likely result in a slower rise in serum sodium than Normal Saline, and therefore have a lower risk of potentiating osmotic demyelination syndrome. Ringer's lactate is therefore recommendable.
- Euvolemic / hyponatremia have an appropriate volume status, and so DO NOT require any particular treatment to defend intravascular volume, and mx should concentrate on preventing worsening hyponatremia
- NOTE; 1. for euvolemic hyponatremic, free fluid restriction (<1L/day) 2. See an appendix B attached for a flow chart.

RAG-Ready Clinical Pathway Narrative:

For HYPONATREMIA, first identify presenting complaint and severity markers from history/exam/investigations. Then apply pathway IF/THEN criteria above. If multiple branches fit, prioritize red flags, hemodynamic risk, and contraindications. Return the next safest management step with one focused clarification question when branch selection is uncertain.

Appendix: n8n Structured Reasoning Contract

Step 1: Condition Identification -> suspected condition, severity clues, missing critical data.

Step 2: Pathway Selection -> branch criteria matched, selected pathway, confidence, needed clarification.

Step 3: Management Response -> next safe action, optional one clarification question, insufficient_evidence flag.

Recommended response JSON:

```
{
  "assistant_reply": "...",
  "next_actions": ["..."],
  "clarification_questions": ["..."],
  "insufficient_evidence": false
}
```